

Office of Enforcement and Removal Operations
ICE Health Service Corps

U.S. Department of Homeland Security
500 12th Street, SW
Washington, D.C. 20536



U.S. Immigration
and Customs
Enforcement

May 25, 2023

MEMORANDUM FOR:

(b)(6),(b)(7)(C),(b)(7)(F) DHSc, FACHE
Assistant Director
ICE Health Service Corps

(b)(6),(b)(7)(C),(b)(7)(F)

THROUGH:

(b)(6),(b)(7)(C),(b)(7)(F) MD
Deputy Assistant Director of Clinical Services/Medical Director
ICE Health Service Corps

(b)(6),(b)(7)(C),(b)(7)(F)

(b)(6),(b)(7)(C),(b)(7)(F) MS
Chief of Staff
ICE Health Service Corps

CDR (b)(6),(b)(7)(C),(b)(7)(F) PA-C
Chief, Investigations Unit
ICE Health Service Corps

(b)(6),(b)(7)(C),(b)(7)(F)

FROM:

LCDR (b)(6),(b)(7)(C),(b)(7)(F) RN
Senior Investigator
ICE Health Service Corps

(b)(6),(b)(7)(C),(b)(7)(F)

SUBJECT:

Mortality Review – Report of Findings

Salvador ROSALES-Vargas, (b)(6),(b)(7)(C)

Executive Summary:

On April 4, 2023, U.S. Immigration and Customs Enforcement (ICE) Health Service Corps (IHSC) received notification of the death of noncitizen Salvador ROSALES-Vargas, (b)(6),(b)(7)(C). (b)(6),(b)(7)(C) ICE assigned Mr. ROSALES-Vargas, a 61-year-old Mexican male, to Stewart Detention Center (SDC), in Lumpkin, Georgia (GA), from February 17 to April 4, 2023, the date of his death. Mr. ROSALES-Vargas's death certificate listed his cause of death as stroke.

Prior to ICE custody, on April 12, 1986, immigration officials admitted Mr. ROSALES-Vargas to the United States as a Lawful Permanent Resident. On February 26, 2010, the United States District Court for the Northern District of Illinois convicted Mr. ROSALES-Vargas for multiple

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offenses, including distribution of cocaine, and sentenced him to 240 months confinement. On August 17, 2021, ICE Enforcement and Removal Operations (ERO) issued Mr. ROSALES-Vargas a Notice to Appear under section 237(a)(2)(A)(iii) of the Immigration and Nationality Act and lodged an immigration detainer. On February 15, 2023, the Bureau of Prisons released Mr. ROSALES-Vargas to ERO Atlanta. ERO Atlanta temporarily booked Mr. ROSALES-Vargas into the Raleigh Hold Room, in Raleigh, North Carolina (NC), and later transferred him to Alamance County Detention Facility, in Graham, NC. On February 17, 2023, ERO Atlanta transferred Mr. ROSALES-Vargas to SDC, in Lumpkin, GA, pending removal proceedings.

On February 18, 2023, during his intake screening, Mr. ROSALES-Vargas reported a medical history of gastroesophageal reflux disease (acid reflux), hypertension (high blood pressure), benign prostatic hyperplasia (enlarged prostate), and an umbilical hernia (portion of intestines protruding through abdominal wall muscles). A registered nurse (RN) completed Mr. ROSALES-Vargas's intake screening and cleared him for general population.

Between February 19 and March 30, 2023, the SDC medical staff provided Mr. ROSALES-Vargas treatment for his preexisting chronic medical conditions and his new diagnosis of hyperlipidemia (elevated cholesterol and triglyceride levels).

On March 31, 2023, at approximately 8:54 p.m., Mr. ROSALES-Vargas experienced syncope (loss of consciousness), and an SDC correctional officer announced the emergency via facility radio. Medical staff responded to the emergency, and at approximately 9:24 p.m., SDC activated emergency medical services (EMS). EMS personnel transported Mr. ROSALES-Vargas to Piedmont Medical Center (PMC), in Columbus, GA, where a physician admitted him with a diagnosis of cerebrovascular accident (stroke).

On April 4, 2023, at 4:52 p.m., a PMC physician declared Mr. ROSALES-Vargas deceased.

Mortality Finding:

As a result of Mr. ROSALES-Vargas's death, the IHSC Assistant Director requested a mortality review to learn from his death by reviewing the care provided and the circumstances leading up to his death. The goal of the mortality review is to determine the appropriateness of clinical care; ascertain whether changes to policies, procedures or practices are warranted; and identify issues requiring further study.

A mortality review committee (MRC) determined Mr. ROSALES-Vargas's medical care at SDC was not provided within the safe limits of practice. Specifically, the MRC determined: 1) SDC did not refer Mr. ROSALES-Vargas to the emergency room (ER) timely; 2) SDC's medical staff did not address Mr. ROSALES-Vargas's sick-call request timely; 3) SDC's medical records and custody log books contained conflicting information; 4) SDC did not communicate pertinent

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information regarding Mr. ROSALES-Vargas's medical condition to EMS and PMC; and 5) SDC did not refer Mr. ROSALES-Vargas to urology timely.

The MRC convened on May 19, 2023, and it consisted of the following IHSC members: Clinical Director (b)(6),(b)(7)(C),(b)(7)(F) MD, physician reviewer; IHSC Chief Advanced Practice Provider (APP) Captain (CAPT) (b)(6),(b)(7)(C),(b)(7)(F) APP reviewer; and Eastern Regional Nurse Manager CAPT (b)(6),(b)(7)(C),(b)(7)(F) nurse reviewer.

The following summary of events was based on: 1) Mr. ROSALES-Vargas's SDC medical records, PMC records, and EMS report; 2) incident and notification reports; 3) ICE ENFORCE Alien Removal Module (EARM) and ICE ENFORCE Alien Detention Module (EADM) database records; 4) Mr. ROSALES-Vargas's SDC detention file; 5) Mr. ROSALES-Vargas's death certificate; 6) video footage from March 31, 2023; 7) an on-site review and staff interviews conducted by Fact Finders Lieutenant Commander (LCDR) (b)(6),(b)(7)(C),(b)(7)(F) CAPT (b)(6),(b)(7)(C),(b)(7)(F) and LCDR (b)(6),(b)(7)(C),(b)(7)(F) at SDC from May 2 – 4, 2023; 8) applicable SDC policies and procedures; and 9) ICE Detention Standards.

Summary of Events

On February 18, 2023, at approximately 12:59 a.m., an RN completed Mr. ROSALES-Vargas's intake screening, documented his medical history and normal vital signs (VS), completed his coronavirus disease 2019 test, which showed negative results, received an order to continue Mr. ROSALES-Vargas's medications (per his transfer summary), referred him to an APP, and cleared him for general population.

On February 20, 2023, an APP completed Mr. ROSALES-Vargas's initial physical examination, documented normal VS, except for an elevated blood pressure reading of 142/82 millimeters of mercury [(mmHg): normal range 90/60 – 120/80 mmHg], ordered baseline laboratory tests, continued his medications, and scheduled a chronic care follow-up.

On February 28 and March 5, 2023, Mr. ROSALES-Vargas submitted sick-call requests for hernia pain. SDC medical staff acknowledged receipt of Mr. ROSALES-Vargas's sick-call requests on February 28 and March 7, 2023, respectively; however, there is no documentary evidence SDC's medical staff conducted a face-to-face evaluation within 24 hours of his first sick-call request.

On March 8, 2023, an APP completed Mr. ROSALES-Vargas's chronic care follow-up, documented normal VS, except for an elevated heart rate (HR) of 107 beats per minute [(bpm); normal range 60–100 bpm], reviewed his laboratory results, which showed an elevated prostate specific antigen [(PSA); prostate cancer screening] and values consistent with hyperlipidemia and prediabetes, and ordered an electrocardiogram (records electrical signals from the heart to identify various heart conditions) and a medication to treat his hyperlipidemia.

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Between March 16 and 30, 2023, SDC's medical staff evaluated Mr. ROSALES-Vargas for his umbilical hernia and complaints of abdominal pain. During those evaluations, SDC's medical staff documented Mr. ROSALES-Vargas's VS, and his systolic blood pressure ranged between 119–136 mmHg, diastolic 76–88 mmHg, and HR 76–88 bpm.

On March 31, 2023, at approximately 5:26 p.m., an RN evaluated Mr. ROSALES-Vargas for his reported symptoms of a “mini stroke” the evening prior and ongoing partial vision loss in his left eye. Neither medical nor correctional staff had awareness of Mr. ROSALES-Vargas's symptoms on March 30, 2023. The RN documented normal VS, clear speech, symmetry with tongue protrusion, equal strength in both arms, and balanced gait. The RN consulted with the on-site APP, who instructed the RN to schedule an APP appointment for April 3, 2023, but there is no documentary evidence to confirm the APP performed a face-to-face evaluation.

- At approximately 8:54 p.m., Mr. ROSALES-Vargas experienced a syncopal episode, and an SDC detention officer announced the medical emergency via facility radio.
- At approximately 9:00 p.m., SDC nursing staff responded to Mr. ROSALES-Vargas's room, evaluated him, and noted right-sided weakness, slurred speech, facial asymmetry, and bladder incontinence. After nursing staff performed an initial assessment, correctional and medical staff transported Mr. ROSALES-Vargas to the medical clinic via stretcher.
- At approximately 9:10 p.m., the RN notified the on-call provider and received an order to transfer Mr. ROSALES-Vargas to the PMC ER via EMS for further evaluation. An SDC central control officer contacted EMS at approximately 9:24 p.m. The SDC medical staff monitored and provided Mr. ROSALES-Vargas treatment while awaiting EMS.
- At approximately 9:35 p.m., EMS personnel arrived at the SDC medical clinic, evaluated Mr. ROSALES-Vargas, documented his right-sided facial droop, right arm and leg weakness, and incomprehensible speech, and departed SDC at approximately 9:50 p.m.
- At approximately 10:33 p.m., a PMC physician admitted Mr. ROSALES-Vargas to the intensive care unit (ICU) with a diagnosis of cerebrovascular accident due to bilateral occlusion of middle cerebral arteries.
- At approximately 10:45 p.m., a PMC physician ordered tissue plasminogen activator [(tPA); medication used to treat strokes by breaking up the blood clots preventing adequate blood flow to the brain], and a PMC RN started the intravenous infusion at 10:56 p.m.

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Between April 1 – 2, 2023, PMC monitored Mr. ROSALES-Vargas in the ICU. Repeat computed tomography scans showed a “large evolving left cerebral hemispheric infarct” (progression of the stroke that affected the left side of Mr. ROSALES-Vargas’s brain).

On April 3, 2023, Mr. ROSALES-Vargas’s status deteriorated. A PMC ICU physician evaluated Mr. ROSALES-Vargas and documented his decreased responsiveness, inability to form speech, fixed and dilated pupils, and the presence of decerebrate posturing (indicative of severe brain damage). After consulting with neurosurgery, the ICU physician documented Mr. ROSALES-Vargas was unlikely to improve and ordered a hospice consult.

On April 4, 2023, at approximately 2:53 p.m., a nuclear medicine brain scan showed the absence of blood flow in Mr. ROSALES-Vargas’s brain.

- At 4:52 p.m., a PMC physician declared Mr. ROSALES-Vargas deceased.

Mr. ROSALES-Vargas’s death certificate revealed his cause of death was stroke, due to or a consequence of hypertension, diabetes, and atherosclerotic vascular disease. His death certificate showed the manner of death as natural.

ICE detention standards used for this review: Performance-Based National Detention Standards (PBNDS), 2011 with 2016 Revisions, and the National Commission on Correctional Health Care (NCCHC) Standards for Health Services in Jails, 2018. The following are identified program strengths, weaknesses, and recommendations found during this review.

Strengths

Strengths identified during this investigation were: 1) SDC’s medical staff promptly ordered an abdominal x-ray and abdominal ultrasound to address Mr. ROSALES-Vargas’s umbilical hernia; and 2) SDC’s medical staff effectively managed Mr. ROSALES-Vargas’s hypertension and ordered medication to treat his hyperlipidemia identified through his baseline laboratory results.

Weaknesses

1. Untimely access to emergency care.

- On March 31, 2023, Mr. ROSALES-Vargas initially reported symptoms of a “mini stroke” at approximately 5:26 p.m.; however, a higher-level provider did not evaluate Mr. ROSALES-Vargas, and SDC’s medical staff did not refer him to the ER for further evaluation until approximately four hours later.
- On March 31, 2023, correctional staff responded to Mr. ROSALES-Vargas’s medical emergency at approximately 8:54 p.m. Per video surveillance footage, medical staff arrived on scene after approximately 5.5 minutes. In addition, SDC did not activate EMS

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until 9:24 p.m., 30 minutes after correctional staff initially identified Mr. ROSALES-Vargas displayed signs consistent with a stroke.

Applicable standards of care for this finding:

- NCCHC 2018 standards: J-D-08, *Hospital and Specialty Care*.
- PBNDS 2011 with 2016 Revisions: Part 4.3, Medical Care; section T. *Emergency Medical Services and First Aid*.
- PBNDS 2011 with 2016 Revisions: Part 4.7, Terminal Illness, Advance Directives and Death; section A. *Terminal Illness*.
- SDC Policy: 13-34, *Medical Emergency Response*.

2. Inappropriate medical care.

- On March 31, 2023, at approximately 5:26 p.m., Mr. ROSALES-Vargas reported symptoms of a stroke, and the evaluating RN consulted with an APP; however, the APP determined Mr. ROSALES-Vargas's plan of care without performing or documenting a face-to-face evaluation.
- On February 27, 2023, Mr. ROSALES-Vargas submitted a sick-call request, and medical staff acknowledged receipt of his request on February 28, 2023, but the nursing staff did not complete a face-to-face encounter within 24 hours.

Applicable standards of care for this finding:

- NCCHC 2018 standards: J-E-07, *Nonemergency Health Care Requests and Services*.
- NCCHC 2018 standards: J-E-09, *Continuity, Coordination, and Quality of Care During Incarceration*.
- NCCHC 2018 standards: J-F-01, *Patient with Chronic Disease and Other Special Needs*.
- PBNDS 2011 with 2016 Revisions: Part 4.3, Medical Care; section S. *Sick Call*.
- SDC Policy: 13-80, *Sick Call*.

3. Conflicting and insufficient documentation.

- SDC's medical staff documented times for key events that conflict with SDC's custody log books, including the time of Mr. ROSALES-Vargas's medical emergency, which differed by one hour, and the time at which SDC contacted EMS.
- On March 31, 2023, an RN evaluated Mr. ROSALES-Vargas for his reports of experiencing "mini strokes"; however, the RN did not document his reported symptoms beyond his partial vision loss. In addition, the RN did not document the time of onset for his partial vision loss, making it difficult to determine whether he met the criteria for tPA.

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Applicable standards of care for this finding:

- PBNDS 2011 with 2016 Revisions: Part 1.1, *Emergency Plans*.
- SDC Policy: 13-34, *Medical Emergency Response*.

4. Inadequate communication.

- There is no evidence SDC relayed Mr. ROSALES-Vargas's entire clinical presentation to EMS and PMC, including the "mini strokes" he reportedly experienced on March 31, 2023, and his initial onset of partial vision loss approximately four hours prior to his syncopal episode. PMC may have chosen a different plan of care had they received complete information about Mr. ROSALES-Vargas's clinical presentation.

Applicable standards of care for this finding:

- NCCHC 2018 standards: J-D-08, *Hospital and Specialty Care*.
- SDC Policy: 13-34, *Medical Emergency Response*.

5. Untimely referrals.

- On March 16, 2023, the SDC medical director denied Mr. ROSALES-Vargas's urology referral and recommended antibiotics to treat prostatitis (inflammation of the prostate) despite his urinalysis results, which did not show results consistent with prostatitis. Mr. ROSALES-Vargas's elevated PSA and presentation warranted a urology referral.

Applicable standards of care for this finding:

- NCCHC 2018 standards: J-D-08, *Hospital and Specialty Care*.
- PBNDS 2011 with 2016 Revisions: Part 4.3, Medical Care; section A. *General*.

Recommendations

- Forward these findings to the IHSC Deputy Assistant Director of Health Care Compliance (DAD-HCC).
- The IHSC DAD of HCC will share these findings through appropriate communication channels to ICE, SDC's administrator and health authority for review and any action(s) deemed appropriate.
- The respective IHSC HCC unit and ICE will ensure the corrective action plan, if applicable, is implemented and sustained.

End of report.